

Arjun Mandloi

Group 3

ENC1101

04 September 2026

A Different Definition of Success

Every couple of years, my family travels to India to reconnect with our relatives, alternating our trips between father's side and my mother's side. This summer, while visiting my mother's side in Lucknow, India, our family stayed two nights with distant relatives. Here, I spent a lot of time with my cousin who is only a year younger than me. During one of our conversations together, she explained her interest in arts. She showed me and my sister her art pieces, spanning from unbelievable realistic paintings to thought-provoking abstract sketches. I remember being surprised upon learning that she never had the option of pursuing her passion of art in her own future. Her father, my uncle, believed that the arts could never provide the stability or success that a typical traditional pathway could. Watching my cousin who was around my age have her passions dismissed made me think about the difference of upbringing in India in comparison to where I lived. Furthermore, this strict, culturalistic view has deep roots in a majority of Indian households which can cause unaddressed mental health issues in India's youth. I began thinking about how in this community, young people are vigorously encouraged to prioritize academic performance based upon family expectations, while their emotional well-being is left unaddressed.

The community of the Indian youth, one which I am a part of, experiences extreme pressure from caregivers in regards to their professional future, which exists alongside the stigmatized issues of mental health. One cross-sectional survey, recently conducted in Karnataka, India in 2024, reveals that a majority of adolescents in the region have high academic stress (86.0%) and experience high parental pressure (87.0%) (Gowda et al., 2024). Furthermore, these factors of stress and pressure lead to diminished mental health quality amongst the youth, as the same study cites how a majority of young Indian adolescents have a low general well-being (57.7%) and around 28.5% have moderately high anxiety in the region (Gowda et al., 2024). This persisting issue of mental health creates another large problem amongst the community: the stigmatization of the youth's mental health. Another study conducted on the Indian youth in 2020 reveals significant levels of stigma associated with mental health problems, with research showing around $\frac{1}{3}$ of India's youth population displaying poor knowledge of mental health alongside negative attitudes towards the issue (Javed et al., 2020). This highlights the community's inability to recognize poor mental health symptoms and how their perception of mental health is viewed negatively. The study also states that this lack of awareness regarding this issue likely stems from a lack of understanding and misinformation about mental health embedded in the Indian youth community (Javed et al., 2020).

This problem ultimately stems from the pressure exerted on India's youth to fit traditional and culturalistic views, pushing adolescents to meet expectations that do not align with their own interests. Looking back at the interaction with my cousin, I have realized that the issue is larger than whether or not someone is able to pursue their career of choice. The consequences of this could range from young people becoming more reluctant to express their emotions and ask for

help, to more extreme mental health issues. My experience in India has shown me that addressing this long-standing issue of parental pressure and mental health stigma can only begin by creating an environment where young people feel comfortable to express their passions and the challenges they face.

References

Pienyu, K., Margaret, B., & D'Souza, A. (2024, December 28). Academic stress, perceived parental pressure, and anxiety related to competitive entrance examinations and the general well-being among adolescents - a cross-sectional survey from Karnataka, India. *Journal of education and health promotion*.

<https://pmc.ncbi.nlm.nih.gov/articles/PMC11756657/>

U.S. National Library of Medicine. (2020, November 13). Stigma associated with mental health problems among young people in India: a systematic review of magnitude, manifestations and recommendations. National Center for Biotechnology Information.

<https://pubmed.ncbi.nlm.nih.gov/33198678/>

Rhetorical Choices

Rhetorical Choice #1: Opening with Personal Anecdote

I began my essay with a real personal experience in order to establish a connection between my community and the issue at hand. This rhetorical choice makes the topic feel personal rather than simply presenting statistics supporting an argument about problems in the Indian youth community.

Rhetorical Choice #2: Evidence Selection

I decided to choose evidence and statistics from two categories: parental pressure and lack of mental health awareness amongst young people in India. This reveals a relationship between the youth community's struggles with familial expectations and consequently their viewpoint/experience with mental health problems. Ultimately, this creates a stronger argument, increasing the scope of the issue at hand while also discussing the severity of the problem.

Rhetorical Choice #3: Organization of the Argument

The order of my essay: personal anecdote to analysis of evidence/statistics to personal reflection. I implemented a personal anecdote to explain my relationship with the issue, and followed up with statistics to show the prevalence of the problem amongst India's youth. Finally, I ended with

a personal reflection in order to reinforce my personal perspective and leave the reader with the central idea that young people in my community should have space to express their passions and interests in order to diminish the problem.